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/ **Practice Exam - 1**

Correct Answer

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Practice Exam - 1

Question 63 of 117



A 55-year-old man with nonischemic cardiomyopathy status post orthotopic heart transplant 5 years prior, with hypertension, dyslipidemia, and type II diabetes mellitus presents with a 3-month history of a raised skin lesion on the right ear. It is nontender, nonpruritic, and has not increased in size over the prior month. He denies fevers, chills, night sweats or weight loss. He is an avid golfer. He was referred to a dermatologist and skin biopsy revealed a 3 mm well-differentiated squamous cell carcinoma (SCC). The lesion was completely excised with negative margins. Current medications include aspirin 81 mg daily, atorvastatin 40 mg daily, tacrolimus 2 mg twice daily (recent trough level 6.7 ng/ mL, goal 4 to 8 ng/mL), and mycophenolate mofetil (MMF) 500 mg twice daily. Vitals in clinic today show blood pressure 130/80 mm Hg, heart rate 90 bpm, and physical examination is unremarkable.

Which of the following is the best next step in management?

- A. Decrease tacrolimus to 1 mg twice daily
- ☒ B. Switch tacrolimus to sirolimus
- C. Decrease MMF to 250 mg twice daily
- D. Continue routine follow-up with dermatologist in one year

Commentary References

Testing Point: The board-certified AHFTC specialist should be able to recognize the appropriate management of chronic immunosuppression following a diagnosis of squamous cell carcinoma.

Correct Answer: B. Switch tacrolimus to sirolimus

Rationale:

Solid organ transplant recipients are at increased risk of malignancy due to chronic immunosuppression, with cutaneous malignancies being the most common post-transplant cancers. Transplant recipients have a markedly increased risk of squamous cell carcinoma (SCC).

Sirolimus and Everolimus have been shown to reduce the incidence and recurrence of SCC compared with continued calcineurin inhibitor therapy, with the greatest benefit seen when conversion occurs early after SCC diagnosis. Therefore, among the available options, conversion from tacrolimus to sirolimus is the strategy most likely to reduce future SCC recurrence.

Answer A: Decreasing tacrolimus alone may lead to suboptimal immunosuppression and does not provide the same reduction in future SCC risk as conversion to an mTOR inhibitor.

Answer C: Although decreasing mycophenolate mofetil may modestly reduce immunosuppression, antimetabolite dose reduction alone has not demonstrated the same reduction in SCC risk as conversion from calcineurin inhibitor therapy.

Answer D: Although this patient has low metastatic risk, a diagnosis of SCC warrants close dermatologic surveillance, typically every 3–6 months rather than annual follow-up, along with counseling regarding rigorous sun protection.

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